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Proposed MHS Internal Medicine Curriculum (4+1 Model)

This schedule reflects an **X+Y (4+1)** alternating block model. In this structure, the academic year consists of ten 5-week cycles per year. For each cycle, residents spend 4 weeks on a core rotation, followed immediately by 1 week dedicated entirely to continuity clinic.

- **Total Cycle:** 10 core blocks (40 weeks) + 10 clinic weeks (10 weeks) + 2 weeks standard vacation/holiday = 52 weeks.
- **Continuity Structure:** For the 2026-2027 academic year, the **PGY-2 class** will complete their "+1" continuity clinic blocks at **NIMA**, remaining at this site to finish out their residency. The **PGY-1** and **current PGY-3** classes are assigned to **CCIM** for their longitudinal continuity site.

In addition to the 5th-week clinic, residents will cycle through dedicated month-long (4-week) ambulatory rotations (such as completing a month in the NIMA clinic) to satisfy ACGME general outpatient requirements and provide staffing support.

PGY-1: Intern Year

Theme: Foundational Patient Care & Skill Acquisition Interns focus heavily on direct inpatient management, primary care foundations, and initial triage skills. * **True Resident Directed Time:** 4 weeks (after 2 weeks required vacation).

Blocks (4-weeks each)	Rotation Types	Specific Rotations Used
4	General Inpatient Wards	Wards B, Wards R, Wards Metro
2	Critical Care (Main)	ICU Main
1	Split: Night Float / Subspecialty	2 weeks Night Float (NF) + 2 weeks Subspecialty Elective
1	Split: Cardiology / Pulmonology	2 weeks Inpatient Cardiology + 2 weeks Pulmonology Consult/Procedural
1	Split: Infectious Disease / Nephrology	2 weeks Infectious Disease + 2 weeks Nephrology
1	Elective / Individualized	Vacation, early subspecialty exposure, board study, or research

PGY-2: Junior Resident Year

Theme: Supervisory Roles, Advanced Triage, & Ambulatory Immersion The PGY-2 takes on team leadership on Wards and gains intensive triage exposure via Emergency Medicine. *Note: Advanced Mechanical Circulatory Support (AMCS) Consults and NIMA are available strictly as electives, whereas*

*Metro ICU can be actively assigned as a core rotation. **True Resident Directed Time:** 6 weeks (after 2 weeks required vacation).*

Blocks (4-weeks each)	Rotation Types	Specific Rotations Used
3	Supervisory Wards	Wards B, Wards R, Wards Metro
1	Critical Care Core	ICU Main
1	Split: Emergency Medicine / Elective	2 weeks Emergency Medicine (EM) + 2 weeks Elective
1	Split: Night Float / Elective	2 weeks Supervisory Night Float (NF) + 2 weeks Elective (e.g., AMCS Consults)
1	Mandatory Geriatrics	Geriatrics
1	Split: Mandatory Consults	2 weeks Neurology + 2 weeks Gastroenterology
1	Split: Pulmonology / Rheumatology	2 weeks Pulmonology + 2 weeks Rheumatology
1	Subspecialty / Elective / Vacation	Additional Consults, Board Prep, Vacation

PGY-3: Senior Resident Year

Theme: Autonomy, Efficiency, & Individualized Education Senior residents increase their efficiency, polish their management, and pursue individualized rotations preparing for their specific career path.

True Resident Directed Time: 10 weeks including 4 week senior track selection (after 2 weeks required vacation).

Blocks (4-weeks each)	Rotation Types	Specific Rotations Used
1	Senior Supervisory Wards	Wards B, Wards R, Wards Metro
1	Senior Track Selection	Junior Hospitalist (Inpatient) OR Primary Care Experience (Outpatient) OR Admitting (Inpatient)
1	Critical Care Core	ICU Main or ICU Metro (Resident's Choice)
1	Split: Cardiology / Procedural	2 weeks Cardiology (Consults OR Clinic) + 2 weeks Procedural Teaching/Supervision (Overseeing PGY-1s on Pulm Consults or in ICU)
1	Hospice & Palliative Care	Hospice and Palliative Care
1	Addiction Medicine	Addiction Medicine

Blocks (4-weeks each)	Rotation Types	Specific Rotations Used
1	Split: Heme-Onc / Emergency Med	2 weeks Hematology-Oncology + 2 weeks Emergency Medicine (EM)
1	Split: Endocrinology / Elective	2 weeks Outpatient Endocrinology + 2 weeks Elective
1	Subspecialty / Elective / Individualized	Elective, Board Prep, or Research
1	Split: Night Float / Elective	2 weeks Night Float (NF) + 2 weeks Elective

Jeopardy & Backup Coverage

To maintain staffing of core rotations, jeopardy coverage will be shared among seniors. To maintain the required 4+1 structure and preserve inpatient caps without compromising continuity clinics or mandatory subspecialty blocks, jeopardy coverage will be assigned to residents on flexible time according to the following rules. Interns are explicitly protected from Jeopardy pulls.

- **Jeopardy Hierarchy:** If an intern calls out, Attendings or Seniors will pick up the patient volume directly.
- **1st Line (PGY-3):** Handled by a PGY-3 currently assigned to an *Elective* or *Consult* block.
- **2nd Line (PGY-2):** If no PGY-3 is available, a PGY-2 on an *Elective* or *Consult* block will be pulled.
- **Final Line (Chiefs):** Chief residents serve as the last line of backup coverage.

Leave & Vacation Policy

- **Rotation Restrictions:** PTO/Vacation is strictly **PROHIBITED** during Wards, ICU, and the "+1" CCIM/NIMA continuity clinic blocks.
- **Blackout Dates:** No PTO is permitted during program transition periods: **June 22 - July 10** and **August 10 - 31**.
- **Structure:** Vacation time must be taken on consecutive days bridged to a weekend (weekends are counted as regular days off, not PTO). Coverage requests require minimum 60-day notice.
- **Leaves of Absence (Medical, Parental, & Caregiver Leave):** The program supports necessary parental, medical, and caregiver leave. Individual scheduling needs and potential impacts on graduation timelines will be addressed on a case-by-case basis with program leadership.

Schedule Adjustments for Required Rotations (Deficit Recovery)

If a resident takes PTO or extended leave during a **mandated** rotation (e.g., Geriatrics or Neurology), the clinical experience must be rescheduled at a later date according to the following rules:

- **The Competency Override (Standard 1-Week PTO):** If a resident takes 1 week of PTO during a 4-week mandatory block, the system does not force a makeup. Completing 3 of 4 weeks satisfies

competency exposure minimums.

- **The Minimum Threshold Flag (Short Blocks):** If PTO reduces a 2-week mandatory split block (e.g., Pulmonology) to 1 week or less, the resident must makeup the time at a future date.
 - **Leaves of Absence Makeup:** During extended leave (e.g., parental/medical), core required blocks will be removed during the absence. Upon return, subsequent pure elective blocks will be utilized to reinstate any missed ACGME mandatory minimums. If insufficient elective time remains, the resident may require a Training Extension.
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ACGME Balance Audit

- **Total Wards + ICU:** ~ 12 months [Min Required: 10]
- **Total ICU:** 4 months exact (PGY1 = 2, PGY2 = 1, PGY3 = 1). [Min 2, Max 6, evenly spread, avoiding PGY-1 exclusivity]
- **Outpatient/Ambulatory:** 44 weeks (~11 months) derived from 30 weeks of +1 clinic, plus mandatory Geriatrics, Endocrinology, Rheumatology, Gastroenterology, and Addiction Medicine blocks. [Meets Min 10 months requirement]
- **Night Float:** 6 total weeks across residency (2 weeks per year in PGY1, PGY2, and PGY3).
- **Subspecialties:** All 9 IM ABIM subspecialties are explicitly mapped within the core block structure.
- **Mandatory Multi-disciplinary:** Geriatrics, Palliative, Addiction, EM, and Neurology are explicitly scheduled. EM is strictly limited to PGY2 and PGY3.
- **Individualized Educational Experiences:** Exactly 6 months (24 weeks). Derived from 20 weeks of true, post-vacation free elective time combined with the 4-week Senior Track Selection. [Meets Min 6 months requirement]